1 ASSESS FIRST

BEFORE MEDICINE
Confirm a patient-specific authorization/order. Check allergy or sensitivity and the last opioid dose, route and time.
ASSESS
Rate the target symptom 0–10 (0 none; 10 worst). Check reversible contributors: pain, urinary retention, constipation, hypoxia, infection, medication effect.
COMFORT
Reposition; reduce noise/light; offer mouth care, calm explanation and a familiar person.
URGENT
Call the hospice clinician now for sudden severe symptom, new focal deficit, suspected sepsis, uncontrolled bleeding or airway threat. Do not delay urgent escalation for routine checks. Reassess after medication: SC/IV 30 min; oral 60 min.

2 CONVERT OPIOID DOSE

USE ONLY WITH THE ORDER
Identify source opioid, route and total administered in the previous 24 h (scheduled + PRN). Apply one factor only:
• Oral morphine → SC morphine: ×0.5
• Oral oxycodone → oral morphine: ×1.5
• Transdermal fentanyl 25 microgram/hour → oral morphine 60 mg/day
Reduce the calculated new opioid by 25–50% for incomplete cross-tolerance or frailty; confirm the prescriber's order. Round DOWN to a measurable 0.1 mL with the listed strength. The table is a calculation aid; it does not create or change an order.
STRENGTH TRANSITION
Through 31 Mar 2027, a facility may hold legacy morphine 5 mg/mL. From 1 Apr 2027 use morphine 10 mg/mL end-state stock. Confirm concentration on the label and use that concentration's volume; never select between two volumes without this check. If stock changes during an episode, recalculate the volume before the next dose.

3 BREAKTHROUGH DOSE

BREAKTHROUGH (PRN)
Use 1/6 of the ordered total 24-hour scheduled opioid dose, same route as ordered, every 1 hour PRN; do not turn PRN into a schedule. Minimum interval: 1 hour.
VOLUME
End-state morphine 10 mg/mL: dose (mg) ÷ 10 = mL; round DOWN to 0.1 mL. Legacy 5 mg/mL through 31 Mar 2027: dose (mg) ÷ 5 = mL. Label the drug, dose, route, interval and concentration together.
REASSESS
SC/IV at 30 min; oral at 60 min. Record offered, administered, response and sedation. Contact hospice if ≥3 breakthrough doses are administered in 4 h or relief is inadequate.

4 TERMINAL AGITATION

RECOGNISE
Terminal agitation = distressing restlessness or delirium with picking, calling out, fearful movements or altered attention in the last days of life. Check reversible contributors (pain, urinary retention, constipation, hypoxia, infection, medication effect) without delaying urgent escalation.
COMFORT
Quiet room; gentle voice; familiar person; explain each step; avoid restraint where possible; provide mouth care.
FIRST LINE
Midazolam 2.5 mg SC PRN, repeat after 1 h if needed; maximum 3 doses in 4 h.
IF PERSISTS
Contact hospice clinician for review and consider haloperidol 0.5 mg SC PRN every 2 h; maximum 2 mg in 24 h, only if ordered. Reassess 30 min after SC dose. If agitation worsens, new rigidity, or paradoxical response occurs: hold further dose and call hospice urgently.

5 MONITOR & ESCALATE

AFTER EACH DOSE
Recheck symptom score, level of consciousness, respiratory rate, airway, distress and response at the source timing: SC/IV 30 min; oral 60 min.
HOLD + CALL HOSPICE
Difficult to rouse, respiratory rate <8/min, new severe myoclonus, or repeated vomiting: hold the next opioid dose and call the hospice clinician.
EMERGENCY
Apnoea, unresponsiveness, cyanosis or airway threat: activate 999, then call hospice. Naloxone, if supplied and ordered, is an adjunct; it does not replace 999 or hospice contact.
CAUTIONS
Renal/hepatic impairment, frailty, or no prior opioid exposure: use the ordered reduction and seek prescriber review before titration.
Contact hospice after ≥3 breakthrough doses in 4 h, persistent agitation after ordered pathway, or inadequate relief.

6 DOCUMENT & HAND OFF

RECORD
Assessment score and observable findings; authorization/order checked; allergy/sensitivity; last dose (drug, route, time); reversible contributors and comfort measures; drug, concentration, dose, route, time and reason; response and reassessment; sedation and respiratory rate; breakthrough count; adverse effects; hospice/prescriber contact and advice.
HAND OFF
At shift change and after escalation, tell the incoming nurse the current symptom, last dose/time, concentration in stock, PRN doses in the count window, response, monitoring findings and agreed plan.
CONTACTS
Routine: Tanneryside Hospice Desk 01632 960 410.
Urgent clinical advice: Elmsworth Hospice Clinician 01632 960 411.
Emergency: 999, then Elmsworth Hospice Clinician.
ELMSWORTH AGRICULTURAL | HOSPICE REFERRAL NETWORK | 2027 v1.0 | Effective 01 Jan 2027 | Approved: Tanneryside Advisory Clinical Governance | England jurisdiction.
Fixed wording: “For use by trained facility nurses with a patient-specific order.”